

FINAL ESSAY

BRINGING HUMANITY TO HELL: BARRIERS OF NON-GOVERNMENTAL
ORGANIZATIONS' PROVISION OF PRIMARY HEALTHCARE TO REFUGEES
IN GREECE, AND BEYOND

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Overview: The Refugee Health Crisis in Greece

The Universal Declaration of Human Rights in 1948 set the foundation for recognizing *health* as a *fundamental human right* for all individuals, irrespective of their nationality or status as refugees.¹ Throughout history, Greece has long served as a principal entry point for refugees. However, a massive volume of forced displacement waves from the Middle East to Europe in recent years has been well documented, due to war and conflicts. The rapid influx of refugees into Greece has emphasized the crucial health issues related to refugees, accompanied by the negative negligent policies implemented by European governments that ignore the duty to protect and care for people. As border closures and pushbacks under the EU-Turkey statement force many refugees to continue their journey underground mainly through the Mediterranean route (see Fig. 1), they are pushed into the hands of human traffickers, exacerbating their vulnerability and placing them at heightened risk of abuse, exploitation, enslavement, and sexual violence.² Even after their arrival, refugees can still experience deprivation, poor living conditions and lack of adequate hygiene due to severe crowding in reception centers, which can make refugees

1. "Universal Declaration of Human Rights," United Nations, <https://www.un.org/en/about-us/universal-declaration-of-human-rights>.

2. Leigh Daynes, "The health impacts of the refugee crisis: a medical charity perspective," *Clinical Medicine* 16(5), 437–40 (2016), <https://pmc.ncbi.nlm.nih.gov/articles/PMC6297302/>.

susceptible to respiratory, gastrointestinal illness and skin diseases.³ This composes a complex landscape of refugee health conditions.



Figure 1. Migrant movements through the Eastern Mediterranean Route.⁴

Hence, to improve all dimensions toward universal health coverage, primary healthcare is crucial as it responds to basic health needs and disease prevention in a whole-society approach.⁵ Amidst the chaos and limbo of the lives of hundreds of thousands of refugees, different Non-Governmental Organizations (NGOs) in Greece strives to offer essential medical services and yet are impeded by numerous barriers that hinder their effectiveness.

3. van Loenen et al., “Primary care for refugees needs, barriers and wishes,” *European Public Health*, Volume 28, No. 1, 82-7 (2017), <https://doi.org/10.1093/eurpub/ckx210>; Joseph et al., “Qualitative Study on Humanitarian Stakeholders' Views,” *Environmental Research and Public Health*, Volume 17, No. 6972. <https://doi.org/10.3390/ijerph17196972>.

4. Dimitter and Lorenz, “Frictions of humanitarian aid in Greece,” *International Humanitarian Action*, Volume 6, No. 1 (2021), <https://doi.org/10.1186/s41018-021-00095-7>.

5. van Loenen et al., “Primary care for refugees needs, barriers and wishes.”

In this essay, we concentrate on a medical charity perspective and attempt to unravel the complexities of this humanitarian crisis with the emphasis on the provision of primary healthcare (PHC) in Greece. The first research question concerns the inherent and external factors that have impacted the primary healthcare provision for refugees, which have driven the demand for service provision by NGOs, including the stability of health systems, bureaucratic red tape, and sociocultural factors. Second, we establish a notion of *effectiveness* that considers the real medical needs of refugees, which is then applied to measure the performance of a renowned international organization *Médecins du Monde* (also known as Doctors of the World). Furthermore, better policies related to its improvements are argued throughout. Thirdly, moral reflections are made to realize the challenges in humanitarian actions, which may help explain stagnant policy changes due to realizability concerns.

Healthcare Provision for Refugees: The Right-Based Approach

The current *right-based* policy implemented by Greece ensures that medical service through the public health system is free of charge for legally documented refugees and asylum seekers (*cf.* Fig. 2). Refugees and asylum claimants are entitled to free services to public hospitals, private doctors, and local health centers with free or low-cost prescriptions, identified by a social security number (PAAYPA or AMKA)

assigned by the government.⁶ However, data has revealed that one in five patients had given up seeking medical care or treatment, struggling to access primary healthcare despite the importance of seeking treatment.

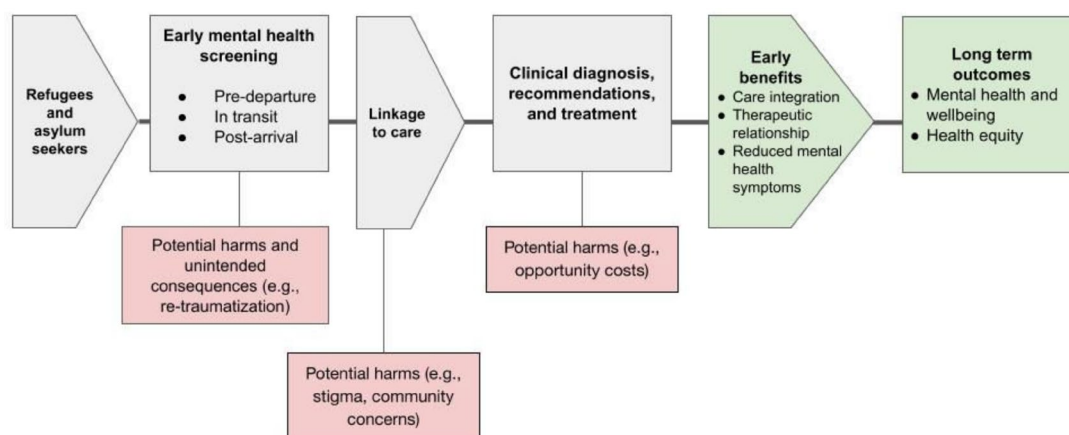


Figure 2. Logic model of health screening along the resettlement pathway.⁷

Time pressure, inadequate entrustment, and lack of information are often mentioned as the biggest barriers for the refugees on the move, for fear of separating from families and the hope for settlement in European destination countries.⁸ This short-stay nature of migrating populations has constantly burdened the trust-building processes in the doctor-patient relationship. In Lesvos, the healthcare system is extremely fragile and understaffed. Medical specialists often demonstrate

6. "Health Insurance," REFUGEE.INFO, <https://greece.refugee.info/en-us/articles/4985624835479>.

7. Magwood et al., "Mental Health Screening Approaches for Resettling Refugees and Asylum Seekers: A Scoping Review," *European Public Health*, Volume 19, 3549 (2022), <https://pmc.ncbi.nlm.nih.gov/articles/PMC8953108/pdf/ijerph-19-03549.pdf>.

8. van Loenen et al., "Primary care for refugees needs, barriers and wishes."

unwillingness to relocate to remote islands, which then burden the existing healthcare workers who are prone to burnout, while overcoming the multi-cultural, multi-religious patient load and the short diagnosis time allowed for disease screening. The sheer volume of refugees has far exceeded the capacity of the National Healthcare System, jeopardizing the quality and reach of healthcare provision.

A person seeking *asylum* is a person who has left their country and *expressed the will* to be recognized as a *refugee*.⁹ Whilst “asylum seekers” and “refugees” are interchangeably used, a refugee must be determined to have met the criteria for leaving the country out of fear of *persecution*. Therefore, NGOs often strive to intervene in this overwhelming situation, by offering essential services. To achieve PHC coverage, NGOs are responsible for filling in the gaps and eliminating the barriers created by the Greek NHS policy, contributing to their unique position in humanitarian actions. Namely, special medical services should be targeted for vulnerable populations that have yet to be documented, people in application for refugee status, and recognized refugees who have encountered barriers when being placed in normal medical settings.

9. Farmakidis, “Refugees and asylum: Concepts-Law-Policy,” (presentation, Hellenic Education & Research Center, Greece, June 10, 2024).

Challenges for NGOs' Provision of Primary Healthcare

Overwhelming Economic Situations and Negative Governmental Responses

In Greece, the healthcare system is a mixed national and social insurance system, funded by taxes and social insurance. In the context of a shrinking economy, local communities may become more sensitive to issues related to healthcare costs. It is noted that public perception regarding out-of-pocket expenditure has contributed to the stigmatization of healthcare access for refugees, while the residents themselves are already facing issues of poverty, material deprivation, and unemployment.¹⁰

Recently, severe staffing shortages in regional hospitals are also reported, with poor responses from private doctors that have declined to assist.¹¹ This has resulted in a higher demand for NGO services even as their own resources are constrained.

However great the medical coverage to refugees is expanded, the demand from minorities is always greater than the actual availability of services.

10. van Loenen et al., "Primary care for refugees needs, barriers and wishes."

11. Newsroom, "Greece ministry decided to address hospital staffing crisis with private doctors," Ekathimerini, <https://www.ekathimerini.com/news/1243755/greece-to-address-hospital-staffing-crisis-with-private-doctors/>.

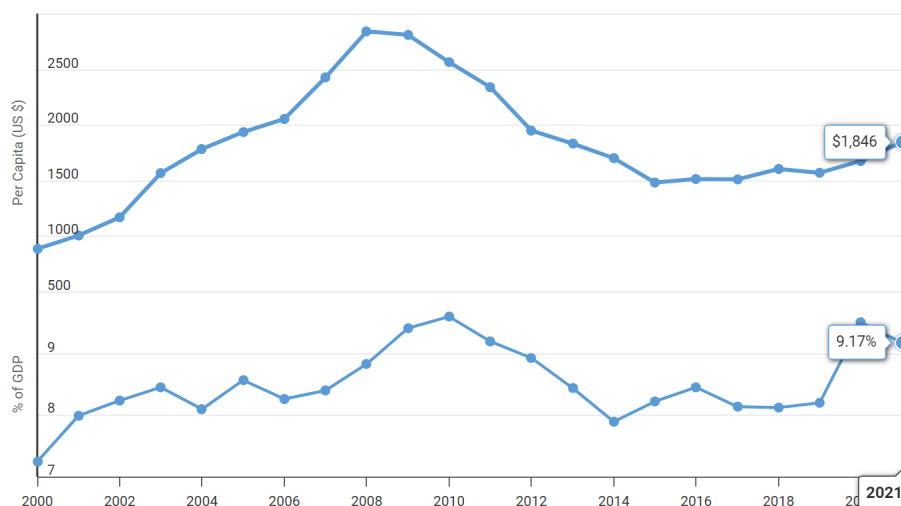


Figure 3. In 2021, Greece spent EUR 1,846 per person on health care, well below the EU average of EUR 3,562, and it was equivalent to 10.9% of GDP in the EU.

Greek government exerts significant influence over the allocation and utilization of funds from worldwide organizations, leading to a concentration of control within the government. Since 2021, the Greek government has taken over the management of the cash assistance programme for asylum seekers in Greece, previously run by the UNHCR. The autonomy of NGOs is impeded by strict regulations and lengthy approval processes, which can delay the delivery of essential resources to vulnerable populations in need. Moreover, NGOs often face budget constraints that result in a scarcity of medical resources, due to the limited funding opportunities and access to national resources by the state. This is unlike public spending on pensions, where a complex set of interrelated factors related to medical staff, competencies, equipment, pharmaceuticals, and facilities are considered.

The burden of excessive administrative requirements, reporting obligations, and compliance procedures further compounds the challenges faced by healthcare-focused NGOs. This administrative overload not only strains organizational capacity but also diverts valuable resources away from frontline healthcare activities, constraining the scalability and impact of healthcare programs that these organizations strive to implement. Furthermore, the need to provide formal documents for justifying their actions has obscured NGOs from developing projects that do not carry immediate values of the government's interests, which means that the funds from the government are largely restricted. This could largely distort the real motivations behind their projects.

Sociocultural Factors

Many Greek healthcare workers are unfamiliar with the religious traditions and customs of refugees, mostly Muslim or Yazidi.¹² During Ramadan, patients may express unwillingness to consume medications during fasting hours, which is an unforeseen situation for doctors. In this *culturally sensitive* medical case, doctors are held accountable for unfamiliar situations that may set chronic patients at risk of experiencing health complications. This complicates the communication process

12. Joseph et al., "Qualitative Study on Humanitarian Stakeholders' Views."

when educating patients about the necessity of maintaining their medication schedule.

Frequently, gender roles have decided the access to healthcare, due to the invasive feelings by patients when being examined by practitioners of a different gender, leading to delays in seeking care. It is hence important for NGOs to maintain the availability of healthcare providers of both sexes, equipped with knowledge of gender sensitivity that creates a safe and respectful environment.

Some refugees also showed high expectations for healthcare with their prior experiences with accessible and high-quality services that secondary healthcare. This disparity can create doubts about the medical advice received, with the preferences of higher prescriptions of antibiotics or refusal to receive vaccine injections that have confused refugees for fairness in medical resource distributions.¹³

Communicating with patients and refugees is of high importance. However, in Lesvos, translators and interpreters are frequently unavailable, which can lead to mistreatment due to the lack of understanding and proper communication. Concerns could be overlooked due to language barriers in the process of communication.¹⁴

13. *Ibid.*

14. Joseph et al., “Qualitative Study on Humanitarian Stakeholders' Views.”; van Loenen et al., “Primary care for refugees needs, barriers and wishes.”

There are no documented, centralized, or standardized procedures when it comes to mental health emergencies of refugees and asylum seekers. It is reported that even for the same hospital on the island, mental health workers can still fail to engage in interdepartmental communication or cross-reference the given diagnoses, exposing a critical weakness and a lack of consideration.¹⁵



Figure 4. Containers and tents that accommodate thousands of refugees in the Lesvos Mavrovouni CCAC. During the summer, the temperature in the tent can reach 40°C without ventilations, and below freezing point without proper heating in winter.

Effective Primary Healthcare: The Case of Médecins du Monde

Mental health emerged as the most common health problem among people residing in migrant camps (56.3%).¹⁶ This is not surprising for refugees, as 3-4% of adult populations affected by emergencies will experience a severe and 15-20% a

15. “For refugees and asylum seekers residing on Lesvos, mental health remains an invisible vulnerability,” International Rescue Committee, <https://www.rescue.org/sites/default/files/2022-06/EN%20Lesvos%20MHPSS%20Advocacy%20Policy%20Note.pdf>.

16. Evangelia Farmakioti et al., “Access to Healthcare Services and Essential Medicines in GREEK Migrant Camps,” *Journal of Immigrant and Minority Health*, Volume 25, No. 3 (2022), <https://doi.org/10.1007/s10903-022-01425-6>.

mild/moderate mental health disorder 12 months after the emergency.¹⁷ The other prevalent health problems are often related to living conditions in reception centers, including prenatal and postnatal care, infectious diseases, gastrointestinal problems, and dental problems at all sites.¹⁸ In order to be *effective*, NGOs should be informed about the *health needs* and *wishes* of the population-under-care. In this section, we shall measure the effectiveness of medical humanitarian aid in Greece, by analyzing the work of the representative NGO, Médecins du Monde (MdM).



Figure 5. A graffiti on the wall at the entrance of the burnt Mória Refugee Camp, which expresses the *wills* of refugees. The Mória Camp has been accused of the poor sanitary conditions and facilities that have triggered the act of self-harm during COVID.

Mission Alignment and Ability

The principle of MdM safeguards *health as a fundamental human right*, aiming

17. Leigh Daynes, “The health impacts of the refugee crisis: a medical charity perspective.”

18. M. Gunst et al., 2018, “Healthcare access for refugees in Greece: challenges and opportunities.”

at provoking an immediate real change in human rights violations. In the Closed Controlled Access Center (CCAC) of Lesbos, MdM often provides treatment for common diseases, and services related to maternal and pediatric health, while psychological and dental care remains non-existent due to the lack of medical staff. However, it is arguable whether MdM referral of the unaddressed issue through the coordination systems that interconnect service providers and agencies is timely and feasible since equitable access to these services is complicated by camp locations in unpopulated areas that lack public transportation, located 5 km away from the center of Mytilene.¹⁹ The transport for both secondary and emergency care can be expensive, which is covered solely by the refugee themselves. This ad hoc system may not always work as populations and organizations who provide support shifts over time. To tackle the issue from root, MdM should also collaborate with local transport providers or secure funding to offer subsidized transport services for refugees requiring secondary or emergency care, ensuring timely access to essential medical services. Nevertheless, an alternative solution is addressed by the other focus on preventative healthcare, including vaccinations, health education, and disease prevention programs that have reduced the prevalence of diseases. Out of the camp,

19. “One Year of Mavrovouni Camp,” Legal Center Lesbos, 2021, <https://legalcentreslesvos.org/2021/09/14/one-year-of-mavrovouni-camp/>.

MdM is able to recognize other vulnerable groups, in addition to asylum seekers. For example, MdM also provides free or minimally charged aid to the homeless and drug addicts in the Open Polyclinic of Athens. This highlights the difference in their capacity of service in and out of the camp, as most of the areas of the polyclinic are addressed comprehensively with the more abundant devices available. Furthermore, the NGO also manages integrated social structures such as mobile clinics in remote areas. It can be believed that their specific scope of vulnerability is based on the archipelagic nature of Greece. It is also evident that the European Union's framework of health standards had set up a baseline that shaped MdM's political actions. This fact is crucial to push for reforms in the government having the prior experiences that can be transferred.

MdM recognizes itself as an autonomous organization, without the interference of government control. In this way, they could receive support from different sectors of society, but at the same time, they could still retain the access to express opinions and push for changes that eventually enhance the lives of vulnerable groups. In the specialized field of refugee healthcare, MdM is equipped with the positive attitude, courage, and professionalism that had leveraged their 13 years of community experiences in refugee camps, as priorities in primary healthcare do not have "one-

size-fits-all” solutions. These experiences have given them the power and resilience to balance the dual roles of being a carer towards the patients and being a gatekeeper of limited resources to prioritize and distribute care, where insights are yielded through everyday practices.²⁰ Their mindset to challenging projects as “not easy but doable” underscores their dedication and moral values to aiding refugees, in the face of adversity, which has mitigated the emergent needs and gained trust and respect from stakeholders, contributing to their positive image to local communities.

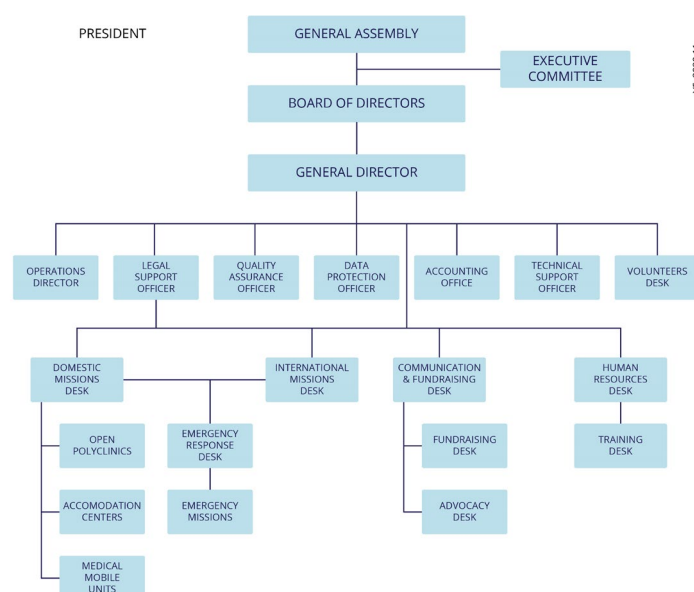


Figure 6. The highly hierarchical organizational structure of Médecins du Monde (Greece).²¹

20. Holmér et al., “How health care professionals handle limited resources in primary care – an interview study,” *BMC Health Services Research*, Volume 23, No. 6 (2023), <https://bmchealthservres.biomedcentral.com/articles/10.1186/s12913-022-08996-y>; Nur Osso and van Houtum, “‘Now You See Me’: Refugees Looking Back at the EU’s Border Camp Watch in Lesbos,” *Geopolitics*, Volume 20, 1-30 (2024). <https://doi.org/10.1080/14650045.2024.2382469>.

21. Médecins du Monde, “Organization Structure,” MdM Greece. <https://www.mdmgreece.gr/en/organisation/structure/>.

MdM has a clear internal structure (as indicated in Fig. 6), separated in their role of work, that could create benefits for designated areas such as medical care, logistics, finance, and advocacy and lead to higher quality services in each domain. In addition, a well-defined internal structure enables MdM to scale its operations efficiently by adding new team members or expanding programs while maintaining clarity in roles and responsibilities. For medical organizations, it is especially important to map specialists who may focus on honing their skills within their specific area of expertise, where their standings can remain solid. However, the hierarchical structure is not without drawbacks. In such a large-scale, internationalized organization, it can be seen that upper management plays an unproportionally bigger role in the decision-making process, which leads to a weakened feeling of coordination and unnecessary delays when encountering unexpected problems.

Working in partnership with local organizations, foundations, institutions, and other NGOs helps MdM establish a strong presence and network within the communities it serves. The deep connections and long-term experiences of MdM could even motivate their peer organizations. MdM has also carried out several volunteering programs aiming to involve healthcare professionals and volunteers in its

missions, at times exceeding 500 people at the peak of the refugee crisis.²² This is beneficial for the holistic, multi-dimensional healthcare that MdM has proposed. Not only does it enhance the organization's operational capacity but also fosters a sense of community and shared purpose. Furthermore, collaborations with diverse donors' portfolios can contribute to MdM's sustainability in the long term by minimizing the risks when relying on funds from a small number of large donors. MdM also values the code of conduct in favor of protecting the information privacy of refugees by refraining unrelated personnel from access, reflecting a level of commitment and respect for *rights* and *dignity*. It can also benefit the trust-building process between refugees and humanitarian organizations, especially when the refugees hold sensitive feelings toward external parties.

Henceforth, while MdM shows promises in the dimensions of enhancing collaborations and networking for national and international initiatives, the potential of sustainable organizational growth by implementing new innovative projects, and its special position in advocating policy changes strengthening the local healthcare systems, the organization still faces threats from the political climate from the hate rhetoric of the government, the impartial coverage of primary healthcare in Lesbos

22. "One Year of Mavrovouni Camp," 2021.

camp, and the weakened coordination due to administrative procedures.

Other Essential Improvements

It is thus necessary to establish sustainable measures in healthcare policies, covering the systematic, managerial, and procedural perspectives. From a systematic perspective, standardized processes and protocols across projects are required for the consistency and quality of care. However, as a comprehensive health project planning guideline made by the international MdM network is already available,²³ MdM should turn to apply this framework to evaluate its existing services and devote to enhancing the transparency of evaluation findings. In this manner, the entire project team will share a common lexicon, frame of reference, and tools that could encourage critical thinking and a questioning attitude. From a managerial perspective, it is important for organizations to deploy suitable project management methods, such as *Agile* or *Scrum* for clearer communication channels and expectations for achieving project goals. For their expressed difficulty in implementing all projects, it is often more important to ensure the quality of services than the quantity of services. From

23. “Health Project Planning: Health Promotion and Humanitarian Action,” Médecins du Monde, 2022, https://www.medecinsdumonde.org/app/uploads/2022/04/MdM_Health-Project-Planning-Guide_2015.pdf.

the procedural perspective, MdM should invest resources to establish ongoing training programs that could better equip medical specialists and healthcare workers with the knowledge of trauma-informed and cultural-sensitive medicine.²⁴ To sufficiently address patients' wishes, a constant review of their key performance indicators accompanied by high standards of compliance with regulations and ethical guidelines that improve refugees' feeling of being accepted, which is the prerequisite of trust-building and a step toward patient-centered approaches.

It is often overlooked that specialized EU agencies are capable of developing and integrating technical assistance even for medical assistance. This should be an area that MdM is capable of advocating for. To ensure that refugees who are on the move will not bear the risk of losing or damaging their medical records, or fear about the confidentiality of their medical diagnosis, digitalized health records can be stored in a transnationally accessible database encoded upon internationally agreed data encryption standards. Via secure mobile applications or digital IDs, portable health records can also be made available to refugees. To address the problems of understaffing in interpretation services, telemedicine could be utilized as long as basic

24. Willey et al., "Racism, healthcare access for people seeking asylum," *Nursing Inquiry*, Volume 29, e12440 (2021), <https://doi.org/10.1111/nin.12440>; Lee and Briggs, "Equity in Healthcare," In *Health Improvement from Hong Kong and Beyond*, edited by B. Y. F. Fong and W. C. W. Wong (Singapore: Springer Nature, 2023), 230, https://doi.org/10.1007/978-981-99-4491-0_15.

network services are sustained to provide seamless communications, unrestricted by remoteness and time. The advent of AI applications in refugee healthcare also provides opportunities for real-time communication between patients and healthcare providers, along with the prediction of health trends and potential outbreaks targeting refugee populations through precision medicine, and the optimization of triage systems for efficiency. Implementing these technological advancements can significantly enhance the quality and accessibility of healthcare for refugees.

Promoting Equality with Good Intentions in “Bad-Bad” Situations

Greece, situated at the epicenter of the humanitarian crisis, and plagued by chronic fiscal crises, has left humanitarians in extreme situations of conflict to provide protection. The act of good intention to save life and preserve human dignity is characterized by two persistent ethical problems, the risk of constituting harm and the legitimization of judicial corruption in the government.²⁵ Has the humanitarian agency, operating amidst actors or systems that deprive people of their rights, perpetuated, or legitimized the violations of human rights?

25. Phillips, “Navigating the Ethical Challenges,” In *Messy Ethics in Human Rights Work*, Vancouver: University of British Columbia Press, 2023, 86, <https://doi.org/10.59962/9780774868525-006>.

In the case of MdM, while they have operated independently of political, religious, and economic interests from a deontological perspective to keep their hands clean from the EU's flawed asylum policies, the reality often necessitates the consequentialism approach. This approach, favored in practice, acknowledges the immediate needs and suffering of refugees, sometimes requiring pragmatic engagement with imperfect systems to achieve the best outcomes. The hybrid approach is crucial to maintain flexibility without compromising core values. By blending principled actions with practical solutions, MdM can navigate complex ethical landscapes and effectively address the urgent needs of those they serve.

Health inequalities are inextricably linked to the social determinants of health, such as socioeconomic, cultural, and environmental conditions. These encompass all areas of human activity: natural environment, food cultivation and production, housing, health services, social networks, education, work or unemployment, gender, age, access to drinking water, and good sanitation and living conditions. Despite the multiple reforms in the Greek NHS, it has remained costly and dysfunctional, impeding universal equality of access.

In discussing of tension between political approaches, Cabot et al. underscored

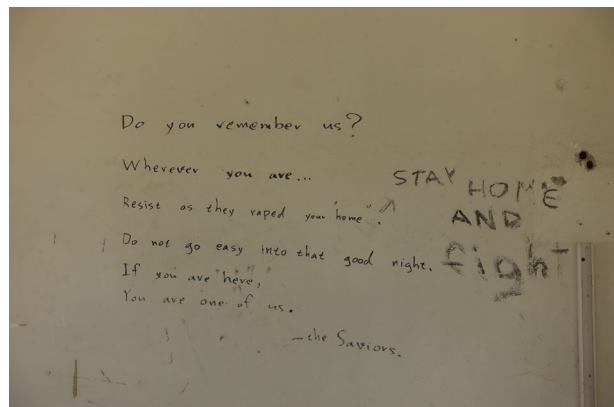
the differences between a *right-based* and a *humanitarian* approach, grounded on entitlement and unstable attitudinal characteristics, respectively.²⁶ The right-based approach enables refugees to assistance and protection owing to their status as human beings whose lives and liberty are under threat. On the contrary, in a humanitarian logic, a refugee must be deemed worthy, deserving, and sufficiently in need of protection. A refugee only becomes a subject of rights only through the capacity to present themselves as “feeble”. In the case of Greece, it is implied that the austerity policies have driven the encroachment of the humanitarian approach into the right-based logic implemented by the Greek government, causing a sense of *objectification*. NGOs thus play an important role in challenging existing human rights violations and government inactions through their experiences that can serve as evidence.

Conclusion

Everyone could bear the possibility of experiencing oppression. Humans have the right to appropriate medical care, regardless of their administrative or social status. Amidst the financial burden and humanitarian crises, humanitarians are left with extreme situations of conflict and deprivations to provide protection. European

26. Cabot, “The European Refugee Crisis and Humanitarian Citizenship in Greece,” *Ethnos*, Volume 84, No. 5, 747-71 (2019), <https://doi.org/10.1080/00141844.2018.1529693>.

governments must take full responsibility to ensure people can access the medical help they need, instead of the reliance on NGOs. They also have a major role in coordinating and guiding NGOs in implementing services that cannot be addressed. Public discourse about migration is invariably highly politicized and often negative, perpetuating a climate that harms refugees' health and well-being. There is still a wide gap left in the ability of NGOs to bring a positive impact on the health of refugees. Now more than ever, medical humanitarian agencies need to unite in defense of the right to provide and receive medical care regardless of income or immigration status.



One death is a tragedy, but one million should not merely be a statistic.

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